



Anil Seth

Easier Healthcare Using IT

This article is not so much about trying out any software as about the need we all feel for reliable care-givers for the elderly and terminally ill. Since this is not something for which one has a regular requirement, it seemed to me that a service that used IT to network with healthcare providers could be immensely valuable.

I was reminded of a start-up in Bengaluru that I had read about some years ago. I made a quick Google search, and through a process of trial and error re-discovered *Babajob.com*. It is a fascinating concept that helps find placements for people in the informal sector. I was thrilled to discover that the site is still operational and in more cities than just Bengaluru.

Hiring from the informal sector

However, the focus has changed somewhat from the original goal of connecting job-seekers from the informal sector with well-off households that require help. As stated at <http://www.techsanganam.com/2011/04/27/a-monster-in-the-making-the-babajob-story/>, "Those behind the original idea realised that people found it easier to hire personnel for offices than for homes. There were many more businesses hiring workers from the unorganised sector than households. Hence, most of Babajob's growth has come from businesses."

My own recent experiences proved to me that there was a need for intermediaries. And since hiring is often a sudden need, there is little time to explore or verify the antecedents of the intermediary, so you can easily be misled. Critical information about a person's track record for reliability and punctuality, aside from the skill-sets, is just not available. A glib talker has a distinct advantage. I would have preferred the intermediary to be a website. The use of ratings on websites like eBay acts as a self-disciplining factor.

A second fact that emerged was that the intermediary is often not a fair paymaster. While you may even pay in advance, there is no assurance that the actual service provider gets paid in time. Holding on to payments seems to be a way of ensuring that the caregiver continues to work with the intermediary.

In a situation in which long-term home nursing is required for the elderly or terminally ill, doctors or the hospital ought to provide some options, but they also lack enough background information to suggest any option with confidence. More often than not, their advice is also based on word of mouth recommendations from their patients.

As our society ages, the need for such services is bound to increase. We have found that there seem to be a fairly large number of people involved in this activity of care-giving/nursing in Chandigarh, which seems obvious since it has the reputation for being a city for the retired. Were such a service accessible to people all over India, family members sitting in Bengaluru, for

instance, could organise care for their old parents in a remote city, even if they cannot rush home during an emergency.

Consulting a doctor

I know just one person who visits a doctor with remarkable frequency. In most cases, the visit to the doctor is delayed until the agony is worse than the agony of going to a doctor! Something does not seem right. An article in <http://techcrunch.com/tag/medlion/> led me to <http://www.medlion.com/> which featured the MedLion service. This began as a reaction against health care becoming very expensive because of medical insurance. However, a significant fact mentioned by TechCrunch website is that "like many direct primary-care practices, more than half their patient interactions are via electronic means, as patients aren't forced by reimbursement rules to visit the office of the care-provider for something that could be done simply over the phone or via email."

Perhaps a home visit by a physician is no longer an option. For critical cases, hospitals may be the better option—but in many cases, an email, SMS, a brief discussion over the phone, or a video call may be adequate to decide the best course of action. Perhaps it can help us move towards the ancient Chinese system of rewarding doctors.

Doctors in Ancient China were paid only as long as their patients were healthy. As soon as the patient fell ill, payment to the doctor stopped! Chinese medicine lays tremendous emphasis on the preventive aspect of medicine (<http://www.wholebodybalance.com/acupunctureflu.htm>).

Given the penetration of mobile phones and the increasing density of smartphones, I hope physicians in India start a similar model. The use of open source applications will make it far easier for the concept to spread.

You should be able to communicate with a doctor, rather than follow the advice of a commercial that was banned, which stated: "You know something is going to be wrong with you, but you don't yet know what it is. Take X and kill it before it hits you!" 

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